Ophthalmic Images

Elevated Retinal Neovascularization in Diabetic Retinopathy

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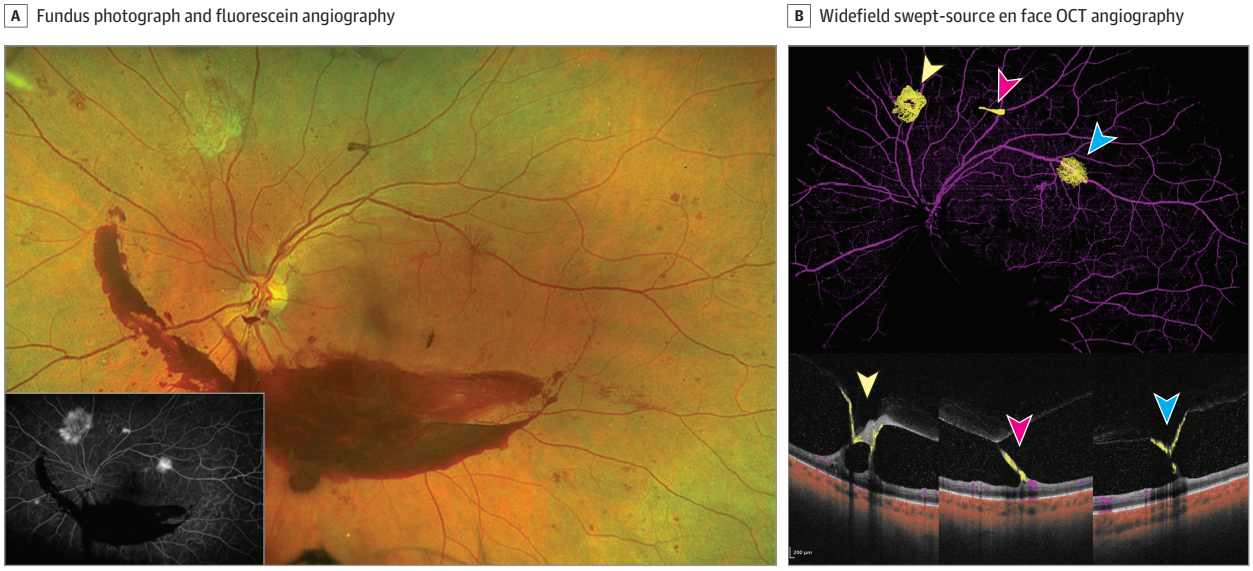


Figure. A, Color fundus photograph with fluorescein angiogram (inset) shows retinal neovascularization with vitreous hemorrhage. B, Widefield optical coherence tomography (OCT) angiography reveals elevated neovascular flow (yellow, pink, and blue arrowheads) extending toward the detached posterior hyaloid. Magenta indicates the retinal slab and yellow the vitreous slab.

A 32-year-old man with type 1 diabetes presented with blurred vision in the left eye. Visual acuity was 20/32 OS. Fundus examination revealed proliferative diabetic retinopathy with vitreous and subhyaloid hemorrhage. Fluorescein angiography (FA) demonstrated multiple foci of retinal neovascularization (RNV) with dye leakage (**Figure**, A). Single-shot widefield swept-source en face optical coherence tomography (OCT) angiography (DREAM OCT [Intalight Inc]) acquired at follow-up after panretinal photocoagulation delineated RNV architecture without leakage blurring the vascular details (Figure, B). A small leaking lesion in the nasal quadrant on FA showed no corresponding abnormal flow breaching the internal limiting membrane. Cross-sectional OCT B-scan with flow overlaid revealed elevated RNV extending toward the detached posterior hyaloid.[1] Compared with FA, OCT angiography requires no injection, avoids dye-related reactions, and may better document presumed traction, but does not assess dye leakage. This case illustrates depth-resolved RNV features, although whether such imaging improves clinical outcomes remains uncertain.[2]

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ARTICLE INFORMATION

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REFERENCES

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